

 MANUFACTURED BY: Midas Care Pharmaceuticals Pvt. Ltd. B-16, MIDC, Waluj, Chhatrapati Sambhajnagar - 431136, India.	may need to be re-instated without delay during periods of stress. It may be advisable to provide such patients with a supply of oral steroid to use in emergency, for example when the asthma worsens as a result of a chest infection. The dose of this product should be increased at this time and then gradually reduced to the maintenance level after the systemic steroid has been discontinued. Discontinuation of systemic steroids may cause exacerbation of allergic diseases such as atopic eczema and rhinitis. These should be treated as required with antihistamine and topical therapy. INTERACTIONS WITH OTHER MEDICAMENTS Beclomethasone is less dependent on CYP3A metabolism than some other corticosteroids, and in general interactions are unlikely, however the possibility of systemic effects with concomitant use of strong CYP3A inhibitors (e.g. cobicistat) cannot be excluded, and therefore caution and appropriate monitoring is advised with the use of such agents PREGNANCY AND LACTATION There is inadequate evidence of safety in human pregnancy. In animals, systemic administration of relatively high doses can cause abnormalities of foetal development including growth retardation and cleft palate. There may therefore be a very small risk of such effects in the human foetus. However, inhalation of beclomethasone dipropionate into the lungs avoids the	high level of exposure that occurs with administration by systemic routes. The use of beclomethasone in pregnancy requires that the possible benefits of the drug be weighed against the possible hazards. The drug has been in widespread use for many years without apparent ill consequence. It is probable that beclomethasone is excreted in milk. However, given the relatively low doses used by the inhalation route, the levels are likely to be low. In mothers breast feeding their baby the therapeutic benefits of the drug should be weighed against the potential hazards to mother and baby. ADVERSE EFFECTS It can cause nausea, cough and give unpleasant taste and smell. Candidiasis of the throat and mouth may develop in some patients, but this can be treated without discontinuation of beclomethasone therapy. Hoarseness may also occur. As with other inhaled therapy, paradoxical bronchospasm with wheezing may occur immediately after dosing. Immediate treatment with an inhaled short-acting bronchodilator is required. This product should be discontinued immediately and alternative prophylactic therapy introduced. Systemic effects of inhaled corticosteroids may occur particularly at high doses prescribed for prolonged periods. These may include adrenal suppression, growth retardation in children and adolescents, decrease in bone mineral density, cataract and glaucoma. Hypersensitivity reactions including rashes, urticaria, pruritus and erythema and oedema of the eye, face,	lips and throat (angioedema) have been reported. TREATMENT OF ADVERSE EFFECT Patients may find it helpful to rinse their mouth thoroughly with water after using the Besomid. As with other inhalation therapy, the potential for paradoxical bronchospasm should be kept in mind. If it occurs, the preparation should be discontinued immediately and alternative therapy instituted. SYMPTOMS AND TREATMENT OF OVERDOSE Acute overdosage is unlikely to cause problems. The only harmful effect that follows inhalation of large amounts of the drug over a short time period is suppression of HPA function. Specific emergency action need not be taken. Treatment should be continued at the recommended dose to control the asthma; HPA function recovers in a day or two. If grossly excessive doses of beclomethasone dipropionate were taken over a prolonged period a degree of atrophy of the adrenal cortex could occur in addition to HPA suppression. In this event the patient should be treated as steroid-dependent and transferred to a suitable maintenance dose of a systemic steroid such as prednisolone. Once the condition is stabilised the patient should be returned to this product by the method recommended above. STORAGE CONDITIONS Store below 30°C. Do not freeze. Avoid storage in direct sunlight or heat. Keep out of the reach of children. Pressurised vial, hence do not puncture	or burn, even when empty. SHELF LIFE 24 Months. PACKAGING Single unit pressurised aluminium canister fitted with metered-dose valve of 200 doses/box. DATE OF REVISION: 17/03/2025	BESOMID 100 MCG/ACTUATION HFA Metered Dose Inhaler (Beclomethasone Pressurised Inhalation BP) NAME AND STRENGTH OF ACTIVE INGREDIENTS Beclomethasone Dipropionate BP – 100 mcg/Actuation. FUNCTION OF ETHANOL BP It is a co-solvent & is also used as a pharmaceutical aid. PRODUCT DESCRIPTION An easily actuated aerosol containing a clear solution, in a suitable pressurized container fitted with metered valve. This is CFC-free metered dose inhaler preparation. The propellant used is HFA-134a. PHARMACODYNAMICS Beclomethasone dipropionate given by inhalation has a potent glucocorticoid anti-inflammatory action within the lungs but does not cause adverse systemic glucocorticoid effects at therapeutic doses. PHARMACOKINETICS Corticosteroids, are in general, readily absorbed from the gastro-intestinal tract. They are also well absorbed from sites of local application. Corticosteroids are rapidly distributed to all body tissues. They cross the placenta and may be excreted in small amounts in breast milk. Corticosteroids are metabolized mainly
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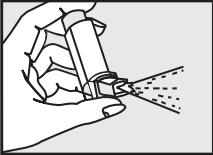
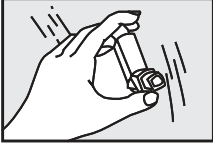
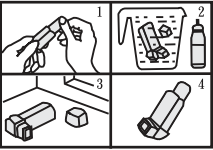
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in the liver but also in the kidney, & are excreted in the urine. The slower metabolism of synthetic corticosteroids with their lower protein binding affinity may account for their increased potency compared with the natural corticosteroids. INDICATIONS It is indicated for the prophylactic management of asthma. RECOMMENDED DOSE The dose should be titrated to the lowest dose at which effective control of asthma is maintained. ADULTS: for maintenance: 1 inhalation (100 µg), three or four times daily or 2 inhalations (200 µg), twice daily. In more severe cases a dose of 600-800 µg (6-8 inhalations) daily is recommended, with subsequent reductions. The maximum recommended daily dose of this preparation is 1 mg. In patients receiving doses of 1500 µg or more daily, adrenal suppression may occur. CHILDREN: 6 years and older-1 inhalation (100 µg), two to four times daily. ELDERLY: No special dosage recommendations are made for elderly patients. INSTRUCTION FOR USE HOW TO USE YOUR INHALER PROPERLY IMPORTANT Follow instructions carefully. Testing your Inhaler	Before using your inhaler for the first time, or its not been used for a week or more, release one puff into the air to ensure the delivery of spray.  1. Remove the mouthpiece cover, and check the mouthpiece critically for cleanliness. Shake the inhaler well.   2. Hold the inhaler as shown. Breathe out gently through the mouth and immediately place the mouthpiece in the mouth.	 3. Close the lips around the mouth piece firmly. Tilt the head slightly backwards, Start breathing in slowly through the mouth. At the same time press the canister as shown, to Release one dose while continuing to breathe in steadily and deeply.  4. Remove the inhaler from the mouth. Hold the breath at least for 10 seconds, or as long as is comfortable. Breathe out slowly.  5. If another dose is required, wait for at least one minute before repeating Steps 1 to 4. After use, replace the Mouthpiece cover.	Note Do not rush stages 2 and 3. It is important to breathe in slowly through the mouth just before Pressing the Canister. To be sure of using your inhaler properly, you may initially practice the steps in front of a mirror. Cleansing  Clean the inhaler at least once a week. - Gently pull the metal canister out the plastic body of the inhaler. Remove the mouthpiece cover. - Rinse the plastic body and the mouthpiece cover in warm water. - Leave to dry in warm place. Avoid excessive heat. - Replace the canister and the mouthpiece cover correctly. CONTRAINDICATIONS Hypersensitivity to beclomethasone is a contra- indication. Caution should be observed in patients with pulmonary tuberculosis. WARNING & PRECAUTIONS Patients should be instructed on the proper use of the inhaler. They should be made aware of the prophylactic	nature of this product therapy and that it should be used regularly at the intervals recommended and not when immediate relief is required. In patients who have been transferred to inhalation therapy, systemic steroid therapy may need to be re-instated rapidly during periods of stress or where airways obstruction or mucus prevents absorption from the inhalation. Systemic effects of inhaled corticosteroids may occur, particularly at high doses prescribed for prolonged periods. These effects are much less likely to occur than with oral corticosteroids. Possible systemic effects include adrenal suppression, growth retardation in children and adolescents, decrease in bone mineral density, cataract and glaucoma. It is important therefore that the dose of inhaled steroid is titrated to the lowest dose at which effective control of asthma is maintained. It is recommended that the height of children receiving prolonged treatment with inhaled corticosteroids is regularly monitored. If growth is slowed, therapy should be reviewed with the aim of reducing the dose of inhaled corticosteroid, if possible, to the lowest dose at which effective control of asthma is maintained. In addition, consideration should be given to referring the patient to a paediatric respiratory specialist.	Prolonged treatment with high doses of inhaled corticosteroids, particularly higher than the recommended doses, may result in clinically significant adrenal suppression. Additional systemic corticosteroid cover should be considered during periods of stress or elective surgery. Patients who have received systemic steroids for long periods of time or at high doses, or both, need special care and subsequent management when transferred to beclomethasone therapy. Recovery from impaired adrenocortical function, caused by prolonged systemic steroid therapy, is slow. The patient should be in a reasonably stable state before being given this product in addition to his usual maintenance dose of systemic steroid. Withdrawal of the systemic steroid should be gradual, starting after about seven days by reducing the daily oral dose by 1 mg prednisolone, or equivalent, at intervals not less than one week. Adrenocortical function should be monitored regularly. Most patients can be successfully transferred to this product with maintenance of good respiratory function, but special care is necessary for the first months after the transfer until the hypothalamic-pituitary-adrenal (HPA) system has sufficiently recovered to enable the patient to cope with emergencies such as trauma, surgery or infections. Patients who have been transferred to inhalation therapy should carry a warning card indicating that systemic steroid therapy
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